WMD / Nerve-Agent Exposure

TE-08 | Toxicology & Environmental

Quick flow

01 Do Not Enter the Hot Zone [EMT / AEMT / Paramedic]

Stage uphill/upwind when possible. Request HazMat, fire, law enforcement, and incident command. Only trained and properly protected personnel may enter the hazard area.

02 Remove + Decontaminate [EMT / AEMT / Paramedic]

Protected personnel remove the patient from the source, remove and bag contaminated clothing, brush dry material, and irrigate as directed. Prevent secondary contamination of responders, the ambulance, and the receiving facility.

03 Airway / Ventilation / Seizure [EMT / AEMT / Paramedic]

Suction secretions, provide oxygen and BVM ventilation, and use the applicable seizure pathway. Do not allow antidote retrieval to delay resuscitation.

04 Cholinergic Syndrome? [EMT / AEMT / Paramedic]

Miosis, salivation, lacrimation, sweating, bronchorrhea, bronchospasm, vomiting, diarrhea, urination, bradycardia, fasciculations, weakness, apnea, or seizure after a compatible exposure.

05 Escalate [EMT / AEMT / Paramedic]

Call Poison Help (1-800-222-1222) and Medical Control. Request HazMat/mutual-aid support and early receiving-facility coordination. Claiborne ground units have no CHEMPACK/Mark I or equivalent atropine/pralidoxime kit.

06 Transport + Handoff [EMT / AEMT / Paramedic]

Transport after indicated decontamination. Communicate agent, route, scene zone, PPE, symptoms, decontamination, airway/ventilatory care, and Poison Help/Medical Control guidance.

Overview

- Use for suspected nerve-agent or organophosphate exposure. Rescuer protection, decontamination, airway/ventilation support, and rapid escalation are the field priorities.
- Claiborne ground units do not carry a CHEMPACK, Mark I, or equivalent atropine/pralidoxime field kit. This protocol does not authorize routine field autoinjector or antidote administration.

Provider-Level Actions

Protect rescuers, stop exposure when safe, support ABCs, and involve Poison Control or Medical Control early.

EMT

- Remain outside the hazard until assigned PPE and training level are appropriate; support removal, decontamination, suction, oxygenation, BVM ventilation, glucose assessment, and rapid transport.
- Recognize the cholinergic syndrome and notify Poison Help, Medical Control, HazMat, and the receiving facility early.

AEMT

- Perform all EMT care plus IV/IO access, authorized fluids, reassessment, and preparation for rapid respiratory deterioration. Do not administer unstocked atropine/pralidoxime or use non-existent autoinjectors under this protocol.

Paramedic

- Perform all prior care plus advanced airway/ventilation, cardiac monitoring, and seizure or shock care under the applicable approved pathway. Coordinate agent-specific therapy with Medical Control and the receiving facility; no CHEMPACK/Mark I field antidote pathway is authorized.

Indications

- Known or suspected nerve-agent release, intentional chemical event, or organophosphate/carbamate exposure with cholinergic findings.

Warnings / precautions

- Do not enter a hot zone, vapor cloud, confined space, or oxygen-deficient atmosphere without required training, monitoring, PPE, and respiratory protection.
- Claiborne ground units do not carry a CHEMPACK, Mark I, or equivalent atropine/pralidoxime kit. Do not use external kit dosing or administer an unstocked antidote.
- Secondary contamination can injure responders and contaminate the ambulance or receiving facility; decontaminate before entry into the clean treatment area.
- Airway suction, ventilation, seizure care, decontamination, Poison Help, Medical Control, and rapid transport take priority over unavailable antidote retrieval.

Clinical pearls

- Atropine and pralidoxime are recognized antidotes for life-threatening cholinergic poisoning, but this Claiborne ground protocol does not create an unstocked field-antidote pathway. Future adoption requires documented kit availability, training, and standing orders.

Related pathways / contacts

Call Poison Help - National Poison Control line

Chemical / HazMat Exposure - Open for zones, PPE, decontamination, and agent cards.

Overdose / Toxic Ingestion - Open for airway, ventilation, and poison-exposure support.

Source and references

Claiborne County EMS approved protocol manual. | Poison Help national line: 1-800-222-1222. | Current Tennessee EMS scope of practice and Claiborne County standing orders control for all provider permissions.

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